

Product X Reimbursement, Pricing, and Payment Pathway Strategy Brief

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NDA-safe prose brief aligned to the Excel model

Audience: reimbursement, market access, and hospital economics stakeholders

Facts reflect the source brief and public CMS materials reviewed for the Excel model.

Why the pathway matters

Product X belongs in the inpatient reimbursement frame, not a Part B drug frame. That makes the Excel model a hospital economics tool: it measures how a bundled payment, a temporary add-on, and a durable DRG adjustment interact across launch and post-launch periods.

The table below captures the minimum set of decisions the brief answers. It keeps the narrative NDA-safe while showing how the workbook translates reimbursement mechanics into pricing and adoption implications.

Core readout	What it means	Excel linkage
IPPS is the governing reimbursement path	The product is a hospital cost, not a Part B drug.	Inputs, Scenario_No_NTAP
NTAP is the bridge	Temporary add-on reimbursement narrows the launch gap.	Scenario_NTAP
DRG recalibration is the durable end state	Long-run adoption depends on bundled payment alignment.	Scenario_Post_DRG, Price_Ladder

1. Reimbursement Foundation: IPPS, Not Part B

Product X sits squarely within the inpatient reimbursement system and should be framed accordingly from the outset. Because the therapy is administered as a 48-hour infusion in the ICU following endovascular thrombectomy, it is reimbursed under the Medicare Inpatient Prospective Payment System, not under Part B drug payment mechanics.

This distinction is foundational. Under Part B, drugs are reimbursed on a per-unit basis, typically tied to ASP, with relatively transparent pass-through economics. None of those dynamics apply here. Instead, Product X is incorporated into a bundled MS-DRG payment, where the hospital receives a fixed amount for the entire episode of care regardless of the therapies used.

The practical implication is that Product X is not reimbursed as a drug. It is treated as a cost within the hospital's DRG payment. The payer does not increase reimbursement when the therapy is used, and the hospital absorbs the full cost at launch. Physician services associated with the admission remain separately reimbursed under Part B, but those payments do not offset or interact with the drug's economics in any meaningful way.

From a strategy perspective, this removes any reliance on traditional buy-and-bill logic and shifts the entire access problem into hospital margin management.

Layer	Payment logic	Commercial meaning	Excel tabs
Part B	Per-unit drug reimbursement tied to ASP-style mechanics	Not the governing launch path for this case	Reference only
IPPS / MS-DRG	Bundled hospital payment for the admission	Hospital absorbs launch cost inside a fixed payment	Inputs, Scenario_No_NTAP
NTAP	Temporary add-on reimbursement on top of the DRG	Bridge from clinical value to hospital tolerance	Scenario_NTAP
ICD-10-PCS	Claim visibility within inpatient procedural coding	Enables utilization tracking and NTAP support	Sources, coding appendix
DRG recalibration	Long-run weight adjustment or DRG split	Durable reimbursement alignment	Scenario_Post_DRG, Price_Ladder

The point of the table is simple: Product X is a hospital economics problem first and a drug payment problem only in a limited sense.

2. DRG Constraint and Economic Misalignment

Under current reimbursement, EVT cases fall into established MS-DRGs with payments generally in the range of approximately \$25,000 to \$50,000, depending on severity. These DRGs were calibrated based on existing standards of care, which do not include a high-cost, continuous infusion therapy such as Product X.

As a result, Product X enters the system as an unrecognized cost within a fixed payment structure. Hospitals must evaluate whether they can absorb the cost through operational offsets such as shorter length of stay or reduced ICU utilization. The available data suggests that Product X can reduce total hospital stay by approximately 3 to 5 days, which is clinically meaningful but economically insufficient to fully offset the therapy at most viable price points.

This creates a structural misalignment. Clinical value accrues across the episode of care, including reduced downstream utilization and improved discharge patterns, but the hospital's financial exposure is immediate and concentrated within the index admission. The payer benefits from reduced total cost of care, while the hospital bears the upfront cost.

This misalignment is the central barrier to adoption at launch and explains why pricing cannot be evaluated independently of reimbursement pathway.

3. Pricing Strategy Anchored to Reimbursement

Pricing for Product X is not set by willingness to pay for clinical benefit in isolation, but by what the hospital can absorb within or alongside the DRG structure.

At launch, in the absence of supplemental reimbursement, hospitals anchor pricing to the recoverable value of reduced utilization. Using reasonable benchmarks for inpatient variable cost, the value of a 3 to 5 day reduction in length of stay translates into approximately \$10,000 to \$15,000. This defines the practical upper bound of an offset-driven price.

As pricing moves beyond that range, the therapy transitions from being economically absorbable to creating negative margin pressure. At approximately \$20,000 to \$25,000, hospitals begin to experience meaningful financial strain, and adoption becomes more selective. Beyond approximately \$30,000 to \$35,000, the therapy is no longer evaluated based on offsets and instead becomes explicitly dependent on additional reimbursement.

This creates a clear inflection point in pricing strategy. Below roughly \$20,000, adoption can be supported through partial offsets and hospital tolerance. Between \$25,000 and \$35,000, adoption requires structured reimbursement support. Above that range, adoption becomes constrained regardless of clinical value unless the payment system evolves.

Approximate price band	How hospitals tend to read it	Adoption implication
Below \$20,000	Offsets can still carry part of the economic burden	Adoption is more feasible without dedicated bridge payment
\$20,000 to \$25,000	Meaningful margin strain begins to appear	Selective adoption and higher committee scrutiny
\$30,000 to \$35,000	Reimbursement support becomes explicit	NTAP or equivalent support is usually required
Above \$35,000	The product is no longer offset-driven	Adoption is constrained until payment changes

The Excel model uses the same logic by separating launch price, reimbursement pressure, and the NTAP bridge into different scenario tabs.

4. NTAP as the Critical Bridge Mechanism

The New Technology Add-On Payment is the single most important mechanism enabling Product X to bridge the gap between clinical value and hospital economics.

NTAP provides incremental reimbursement on top of the DRG, typically covering up to approximately 65 percent of the cost of the new technology. While this does not fully eliminate the hospital's financial exposure, it materially reduces it. A therapy priced at \$30,000, for example, would expose the hospital to the full cost at launch but only

approximately \$10,000 after NTAP support. This shifts the evaluation from a clearly negative margin to a manageable one.

Equally important, NTAP functions as a signal of validation from CMS. Approval indicates that the therapy demonstrates substantial clinical improvement and that its costs are not already captured in existing DRGs. This reduces payer skepticism and facilitates internal hospital decision-making, particularly within value analysis committees.

However, NTAP is temporary. It typically applies for two to three years, after which reimbursement reverts to the DRG unless adjustments are made. This creates a defined window in which Product X must demonstrate real-world utilization and cost impact sufficient to support permanent DRG recalibration.

From a strategy standpoint, NTAP is not the endpoint. It is a bridge that enables adoption while generating the data required for long-term reimbursement alignment.

5. ICD-10-PCS Coding as a Prerequisite for Payment Visibility

A critical but often underappreciated component of the reimbursement pathway is ICD-10-PCS coding. In the inpatient setting, drugs are not tracked or reimbursed through NDC-based mechanisms. Instead, technologies must be identifiable within the procedural coding system to create visibility for utilization and cost.

For Product X, this likely involves securing a New Technology ICD-10-PCS 'X code.' These codes are used to identify new therapies that are not yet reflected in DRG definitions. They allow hospitals and CMS to track when the therapy is used, how frequently it is used, and what incremental costs are associated with it.

This coding step is essential for two reasons. First, it supports NTAP eligibility by demonstrating that the therapy is distinct from existing care and not already captured in DRG payments. Second, it enables CMS to collect the data required to recalibrate DRG weights or create new groupings in the future.

The 48-hour infusion profile of Product X strengthens this pathway. Because it is a prolonged, protocol-driven intervention rather than a single administration, it creates a clear procedural and resource-use signal. This makes it easier to define, code, and differentiate from standard supportive care.

Without appropriate ICD-10-PCS coding, Product X risks being invisible within claims data, which would undermine both NTAP qualification and long-term reimbursement evolution.

6. DRG Evolution and Long-Term Reimbursement

The long-term reimbursement pathway for Product X follows a standard CMS progression but requires successful execution at each stage.

At launch, the therapy is incorporated into existing DRGs and functions as a cost center. With NTAP approval, hospitals receive partial reimbursement, enabling broader adoption. During this period, CMS collects real-world data on utilization, cost, and outcomes.

If Product X demonstrates sustained use and materially changes the cost structure of the episode, CMS may respond in one of two ways. The more common outcome is a reweighting of existing DRGs to reflect higher average costs. In some cases, if the therapy defines a clinically distinct subgroup, CMS may create a DRG split.

This transition is critical. NTAP is temporary, but DRG adjustment is permanent. Without it, the therapy reverts to being under-reimbursed once NTAP expires.

From a strategic perspective, the objective is to ensure that Product X becomes embedded in standard care pathways quickly enough, and with sufficient economic signal, to justify DRG recalibration before NTAP sunsets.

7. Strategic Synthesis

Product X operates within a reimbursement environment where clinical value, payer benefit, and provider economics are not naturally aligned. The therapy improves outcomes and reduces downstream costs, but the hospital absorbs the upfront financial burden.

As a result, the success of Product X depends on sequencing and execution across four interconnected levers:

- Pricing discipline, aligned to what can be supported within DRG constraints
- ICD-10-PCS coding, to establish visibility and differentiation
- NTAP approval, to enable early adoption and reduce financial risk
- DRG recalibration, to create durable reimbursement alignment

The key insight is that Product X does not fail or succeed based on clinical performance alone. It succeeds if it can transition from a bundled cost to a recognized component of the inpatient payment system within the NTAP window.

Failure to achieve that transition results in a structurally constrained product, regardless of clinical differentiation.

Appendix A. Workbook Map

Workbook tab	What it answers	Why it matters for the brief
Inputs	Launch price, DRG anchors, LOS assumptions, and NTAP rate	Defines the base scenario set
Episode_Cost_Break down	Bundled and non-bundled episode components	Separates hospital margin from payer-visible spend
Scenario_No_NTAP	Hospital absorption with no bridge reimbursement	Shows the launch-period risk
Scenario_NTAP	Bridge economics under policy-consistent NTAP	Shows the transitional effect
Scenario_Post_DRG	End-state after DRG reweighting	Shows the durable reimbursement state
Episode_Total_Care	Whole-episode spend from the payer lens	Shows downstream savings beyond the hospital bill
Sensitivity	How price, delay, and revenue change the outcome	Shows where the model is resilient or fragile
DRG_Weights_Trend	Historical DRG movement	Gives directional context for reimbursement evolution
DRG_YoY_Change	Year-over-year change by DRG	Shows trend behavior rather than a single point
DRG_Definitions	Comparator group titles	Anchors the analysis in specific DRG language
Price_Ladder	Scenario sweep across price points	Illustrates the practical inflection range

Workbook tab	What it answers	Why it matters for the brief
Episode_Scenarios	Conservative, base, and optimistic assumptions	Shows the range of utilization effects
Sources	Public source list	Keeps the brief traceable without exposing internal material

This appendix is the companion guide to the Excel file and is written to stay NDA-safe: it describes the workbook structure without revealing internal source paths or client-only terminology.

Appendix B. Public Sources

Source	URL
CMS NTAP page	https://www.cms.gov/medicare/payment/prospective-payment-systems/acute-inpatient-pps/new-medical-services-and-new-technologies
CMS Acute Inpatient PPS overview	https://www.cms.gov/medicare/payment/prospective-payment-systems/acute-inpatient-pps
CMS IPPS guide for medical technology companies	https://www.cms.gov/cms-guide-medical-technology-companies-and-other-interested-parties/payment/ipp
CMS ICD-10-PCS code request process	https://www.cms.gov/medicare/coding-billing/icd-10-codes/process-for-requesting-new-revised-icd-10-pcs-procedure-codes
CMS outlier payment overview	https://www.cms.gov/Medicare/Medicare-Fee-for-Service-Payment/AcuteInpatientPPS/outlier.html
CMS Physician Fee Schedule overview	https://www.cms.gov/cms-guide-medical-technology-companies-and-other-interested-parties/payment/physician-fee-schedule

Public sources are included for traceability only. The narrative itself is written to avoid quoting internal or confidential material.